HealthConnect Clinic

Project Summary — Project Management

1. The Specific Problem This Work Addresses

Establishing the project management foundation for a standalone AI-powered no-show reduction and patient support initiative at HealthConnect Clinic: defining scope, authority, timeline, resourcing, risk, dependencies, and communication for the project as a whole, independent of any other track or party.

2. Resources Used

- The HealthConnect business scenario and central project question, used to define the problem, objectives, and scope.
- A structural-level understanding of the HealthConnect Appointment Dataset and Clinic Knowledge Base — enough to plan data-related work packages, dependencies, and risks, without duplicating detailed technical analysis.
- Standard project management practice for charters, work breakdown structures, risk and dependency registers, and communication planning.

3. Key Observations

- The project has real sequencing dependencies even though it is managed as a single, self-contained initiative: data preparation must complete before modelling, modelling before deployment, and the Knowledge Base scope must be finalised before the assistant can be designed.
- A 20-week active schedule needs an explicit exclusion for December, or the plan looks achievable on paper but fails against real-world office closures and public holidays.
- Because the assistant and prediction model could be perceived as making clinical judgements, safety boundaries and a phase-gate sign-off structure need to be built in from the start, not added later.

4. Proposed Approach

A Project Charter was developed defining the business case, scope (in/out), Project Manager and Sponsor authority levels, methodology, high-level communication approach, assumptions, constraints, resourcing, high-level risks, and success criteria. This is supported by five separate spreadsheet registers — Stakeholder Register, Work Breakdown Structure, Risk Register (with an accompanying Assumptions, Limitations and Risk Summary), Communication Plan, Detailed Timeline, and Dependency Register — each built to track its own type of information without duplicating the others.

5. Key Considerations Affecting the Project

- Data quality and identifier consistency in the appointment dataset could constrain both the model's predictive value and the integration work package.
- The AI assistant must stay strictly within the approved Knowledge Base scope to avoid unsafe or out-of-scope responses.
- The holiday-adjusted schedule leaves limited slack; the dependency chain means a delay in one work package can ripple into later ones.
- POPIA compliance applies to anonymised, health-related data with the same seriousness as it would to real patient data.

6. Proposed Focus for Week 5

Week 4 covered the Initiating process group: confirming the business problem, defining scope, and producing the Charter, Stakeholder Register, and initial (high-level) registers. Week 5 moves the project from

Initiating into Planning — building out the detailed plans and baselines that Execution will run against, rather than starting execution itself:

- Obtain formal Sponsor sign-off on the Project Charter and Scope Statement, formally closing Initiating.
- Develop the detailed project schedule from the WBS: sequence all activities, confirm logical dependencies, and identify the critical path.
- Conduct schedule risk analysis on that critical path — identify which activities have the least float (e.g., the model/assistant build spanning the December recess), and size a schedule contingency/buffer around them rather than assuming the plan holds exactly as drafted.
- Baseline the schedule, scope, and budget so that Week 6 onward can be tracked as variance against a fixed reference point, with any change routed through change control.
- Expand the Risk Register from high-level risks into a fuller planning-stage set (adding probability/impact detail and named owners) and finalise the Resource Management Plan with confirmed, named resources in place of role placeholders.